

Passive Euthanasia in India: The Gap Between Legal Recognition and Practical Implementation

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ABSTRACT

Passive euthanasia, understood as the withholding or withdrawal of life-sustaining medical treatment so as to permit a patient to die naturally, has emerged as one of the most complex and contested issues in Indian constitutional and medical jurisprudence. The decision of the Supreme Court of India in *Common Cause (A Regd. Society) vs Union Of India* on 9 March, 2018 marked a significant constitutional development by recognizing passive euthanasia as legally permissible and affirming the right to die with dignity as an integral facet of Article 21 of the Constitution. However, despite such legal recognition, the actual implementation of passive euthanasia in India continues to remain highly restrictive, inconsistent, and judicially cautious. This reveals a fundamental tension between constitutional recognition in principle and practical accessibility in adjudication.

This paper examines the evolution and judicial application of passive euthanasia in India through a doctrinal analysis of major judicial decisions, namely *P.Rathinam vs Union Of India*, *Smt. Gian Kaur vs The State Of Punjab* on 21 March, 1996, *Aruna Ramchandra Shanbaug vs Union Of India & Ors* on 7 March, 2011, *Common Cause (A Regd. Society) vs Union Of India* on 9 March, 2018 and *Harish Rana vs Union Of India & Ors.* on 2 July, 2024 one of the most recent and significant developments in this field. The paper studies the factual circumstances of these cases, the legal questions involved, the reasoning adopted by the courts, and the conditions under which passive euthanasia has been either permitted or denied. In doing so, it seeks to understand how Indian courts determine eligibility for passive euthanasia and what judicial, constitutional, and medical considerations shape such decisions.

The paper further traces the historical development of passive euthanasia in India, including the period prior to its legal recognition, the constitutional and ethical factors that contributed to its acceptance, and the significance of its continued relevance in contemporary Indian law. Special emphasis is placed on *Common Cause* and *Harish Rana* as leading decisions that illuminate both the normative foundation and the practical limitations of the Indian passive euthanasia framework. The study argues that while passive euthanasia has been recognized in law, its application remains constrained by vague eligibility standards, procedural safeguards, judicial hesitation, and concerns relating to misuse, medical uncertainty, and institutional accountability. The paper identifies a significant gap between legal permissibility and effective implementation and concludes by examining the major doctrinal and practical challenges that continue to shape the future of passive euthanasia jurisprudence in India.

Keywords: Passive euthanasia, right to die with dignity, Article 21, end-of-life decision-making, constitutional morality, medical autonomy, judicial review, living will, *Common Cause*, *Harish Rana*.

I. Introduction

Concept and Meaning of Passive Euthanasia with Judicial Development Through Landmark Cases

Euthanasia¹ refers to the act or practice of painlessly putting to death persons suffering from a painful and incurable disease or an incapacitating physical disorder, or allowing them to die by withholding treatment or withdrawing artificial life-support measures. In most legal systems, due to the absence of a specific statutory provision, it is generally regarded either as suicide when performed by the patient himself or as homicide when performed by another. However, medical jurisprudence recognizes that physicians may lawfully decide not to prolong life in cases of extreme suffering and may administer drugs to relieve pain even if such treatment incidentally shortens life. Within this broader understanding, passive euthanasia, which involves withholding or withdrawing life-sustaining treatment to allow a patient to die naturally, did not originally emerge as a direct legal issue but gradually entered constitutional discourse in India through judicial interpretation under Article 21 of the Constitution, particularly in relation to dignity, autonomy, and the limits of artificial prolongation of life.

A significant early reference to this evolving concept can be traced to *P. Rathinam v. Union of India*,² where the primary issue concerned the constitutional validity of Section 309 of the Indian Penal Code dealing with attempt to commit suicide. Although the case did not directly adjudicate upon euthanasia, the Court drew an important distinction between suicide and euthanasia, observing that euthanasia, particularly passive euthanasia, is not entirely unrelated to ending one's life, especially in situations involving the consent of the patient or their relatives. The Court linked this reasoning to Article 21 by interpreting the right to life as encompassing not merely survival but a life with dignity, and it suggested that such a right may also carry a negative dimension, namely the right not to continue living under conditions of extreme suffering. This reasoning, although later overruled, opened the conceptual space for recognizing that in certain circumstances continued existence may involve prolonged suffering with no reasonable prospect of recovery, thereby laying an early intellectual foundation for the development of passive euthanasia in Indian constitutional law.

The conceptual and doctrinal foundation was further clarified in *Gian Kaur v. State of Punjab*³, where a Constitution Bench of the Supreme Court re-examined the scope of Article 21 and overruled the decision in *Rathinam*. The case arose in the context of a constitutional challenge to Sections 306 and 309 of the Indian Penal Code, which criminalize abetment of suicide and attempt to commit suicide. The Court categorically held that the "right to life" under Article 21 does not include the "right to die," thereby upholding the constitutional validity of both provisions. However, in doing so, the Court introduced a nuanced and significant distinction between suicide as an unnatural termination of life and the process of dying with dignity in cases of terminal illness. It observed that the right to life includes the right to live with human dignity, and such dignity may extend to the natural process of death when life is gradually ebbing out. The Court further clarified that in such situations, allowing the process of natural death to proceed cannot be equated with causing death. This reasoning implicitly recognized that artificial prolongation of life through medical intervention, where recovery is impossible, may undermine dignity rather than protect it. Thus, while maintaining that any act intended to cause death remains punishable, the Court laid the normative and jurisprudential groundwork for distinguishing such acts from passive euthanasia, which involves withdrawal or withholding of life-sustaining treatment in cases of irreversible or terminal conditions. Although passive euthanasia was not expressly legalized, the judgment provided the constitutional legitimacy for a dignified death and became the foundation for subsequent legal developments.

¹ ISSN - 2583-2344

² 1994 AIR 1844, 1994 SCC (3) 394

³ 1996 SCC (2) 648

The issue of euthanasia came directly for judicial determination in *Aruna Ramachandra Shanbaug v. Union of India*,⁴ where the Supreme Court was confronted with fundamental questions regarding whether the right to life includes the right to die, the legality of euthanasia, the distinction between active and passive euthanasia, and the conditions under which passive euthanasia may be permitted. The case arose from a petition seeking withdrawal of life-sustaining treatment of a rape victim who had suffered severe brain damage and remained in a persistent vegetative state (PVS) for several decades. The Court undertook a detailed medical and factual assessment of her condition and observed that although her cortex was severely damaged, her brainstem was functional. She was able to breathe on her own without a ventilator, digest food, and perform involuntary bodily functions. Visual evidence showed that she could make sounds, blink, and swallow food. On this basis, the Court concluded that she could not be regarded as clinically or legally dead. Although the possibility of recovery was extremely remote, the Court held that passive euthanasia would not apply in her case because she was not dependent on artificial life-support systems and her survival was not sustained by extraordinary medical intervention. The Court reasoned that where there exists even a minimal possibility of survival in a stable condition, and where the patient is not completely dependent on artificial support, passive euthanasia cannot be permitted. Consequently, the plea for withdrawal of feeding and treatment was rejected. At the same time, the Court made an important distinction between active euthanasia, which involves a direct and intentional act to cause death and remains illegal, and passive euthanasia, which may be permissible in exceptional circumstances subject to strict safeguards and judicial approval. It also laid down procedural guidelines requiring approval from the High Court, thereby introducing a regulated mechanism to prevent misuse and protect the interests of the patient, family, and medical practitioners.

The development of the law on passive euthanasia in India was also influenced by comparative jurisprudence, particularly the decision in *Airedale NHS Trust v. Bland*⁵, where the House of Lords held that withdrawal of artificial life-support from a patient in a persistent vegetative state does not amount to an act of killing but rather an omission to continue treatment that no longer serves any therapeutic purpose. The decision emphasized the distinction between actively causing death and allowing a patient to die naturally, which significantly shaped the reasoning adopted by Indian courts in subsequent cases.

Finally, the legal position was conclusively settled in *Common Cause v. Union of India*⁶, where a Constitution Bench of the Supreme Court comprehensively examined the earlier decisions including *Aruna Shanbaug*, *Gian Kaur*, *P. Rathinam*, and *Airedale NHS Trust v. Bland*. The Court addressed all major questions relating to euthanasia, including whether passive euthanasia requires legislative support, the distinction between passive and active euthanasia, the legality of passive euthanasia, and the guidelines and safeguards necessary to protect the patient, family, and doctors. The Court held that passive euthanasia is legal in India as it does not involve actively causing death but merely allows the natural process of death to occur in cases where there is no hope of recovery and continuation of treatment would only prolong suffering. It clarified that passive euthanasia does not necessarily require legislative enactment and can be recognized through constitutional interpretation under Article 21. The Court reaffirmed that while there is no fundamental right to die, the right to die with dignity is an integral part of the right to life in cases of terminal illness where the process of natural death has already commenced. It also laid down detailed safeguards, including recognition of advance directives or living wills, constitution of medical boards, certification by competent doctors, and procedural oversight mechanisms to ensure that decisions are taken in the best interest of the patient and free from misuse. These

⁴ AIR 2011 SUPREME COURT 1290

⁵ 1993] AC 789

⁶ AIR 2018 SUPREME COURT 1665

safeguards were designed to protect the autonomy and dignity of the patient, prevent exploitation by family members, and provide legal protection to medical practitioners making such decisions.

Thus, through a sequential judicial development beginning with conceptual observations in *P. Rathinam*, doctrinal clarification in *Gian Kaur*, conditional recognition in *Aruna Shanbaug*, and final constitutional affirmation in *Common Cause*, passive euthanasia has been recognized as legally permissible in India subject to strict safeguards. The law clearly distinguishes it from active euthanasia, which remains prohibited, and reflects a shift towards a more humane and dignity-oriented understanding of life and death within the constitutional framework.

Difference between Active and Passive euthanasia ⁷

Euthanasia means the deliberate act of ending the life of an individual to relieve him or her of prolonged pain and suffering caused by serious illness or incurable medical conditions. The word euthanasia is derived from the Greek term meaning “good death.” In medical and legal discussions, euthanasia generally applies to patients who are in a terminal condition, permanently unconscious, or facing unbearable suffering with no reasonable chance of recovery. Globally, the laws relating to euthanasia differ widely, as some countries permit certain forms of assisted dying under strict conditions while others completely prohibit it. In India, euthanasia has been explained and differentiated mainly through judicial decisions such as *Aruna Ramachandra Shanbaug v. Union of India*, *Common Cause v. Union of India* and *Gian Kaur v. State of Punjab*, where the courts have clarified its types and legal position.

Active euthanasia ⁸refers to the intentional act of causing a person’s death with the objective of relieving suffering. This may be done by administering a lethal dose of medication or by any medical procedure that directly results in death. The essential feature of active euthanasia is that it involves an active intervention by a doctor or any other person to end the life of the patient. It allows individuals to have control over their own life and death, especially in cases where they are suffering from unbearable pain or terminal illness. Supporters argue that it is a compassionate choice that respects autonomy and dignity, as it provides a quicker and more certain death and spares the person from prolonged suffering. However, active euthanasia raises serious concerns regarding its misuse and the possibility of a slippery slope. It may lead to involuntary euthanasia or devaluation of human life. Many ethical and religious perspectives consider it morally wrong because it involves intentionally causing harm or directly taking a life.

Passive euthanasia, on the other hand, refers to withholding or withdrawing treatment or life-sustaining measures with the intention of allowing a person to die naturally. Unlike active euthanasia, it does not involve a direct act to cause death but allows the natural progression of the disease to take its course. One of its key features is that it follows the principle of double effect, where the primary intention is to relieve suffering and not to cause death, even though death may be a foreseeable consequence. For example, when a terminally ill patient is in extreme pain and no effective treatment is available, doctors may decide to stop further treatment to reduce suffering, even if it results in death. Supporters consider passive euthanasia to be more morally acceptable as it respects the principles of non-maleficence and sanctity of life, and allows a more natural death process where the patient can spend remaining time peacefully. However, critics argue that it may prolong suffering unnecessarily and involves difficult decisions regarding continuation or withdrawal of treatment, placing a burden on doctors and family members.

In active euthanasia, the emphasis is on an individual’s right to make decisions about their own life and death. Supporters view it as a dignified and compassionate option, while opponents raise concerns about misuse, ethical

⁷ Active and Passive Euthanasia by James Rachels (1975)

⁸ active-euthanasia-vs-passive-euthanasia

issues, and the possibility of intentional harm. In passive euthanasia, the focus is on allowing natural death while respecting the sanctity of life. Supporters consider it more acceptable as it does not involve direct killing, whereas critics argue that it may not always be in the best interest of the patient and can lead to prolonged suffering.

According to Indian law and judicial interpretation, passive euthanasia is legal in India, whereas active euthanasia is not. In cases like *Aruna Ramachandra Shanbaug v. Union of India*, *Common Cause v. Union of India* and *Gian Kaur v. State of Punjab*, the courts have consistently held that a person's life cannot be actively taken if there is any hope of survival. However, passive euthanasia has been recognized as part of Article 21, which includes the right to life and personal liberty along with the right to dignity, autonomy, and the right to refuse treatment. Therefore, when the court permits passive euthanasia, it considers that there is no reasonable hope of recovery and that continuing treatment would only prolong suffering and undermine the dignity of the patient.

In comparison, active euthanasia remains illegal in India and in most countries because it involves directly causing death, which amounts to murder or culpable homicide. Even if it is done with the intention of reducing pain and suffering, the law does not permit such an act. In *Gian Kaur v. State of Punjab*, the Court clearly held that the right to die is not a fundamental right, and therefore a person cannot take his own life or seek assistance in doing so. Similarly, in *Aruna Ramachandra Shanbaug v. Union of India*, the Court expressed concern that permitting active euthanasia in India could lead to misuse, particularly in cases involving property disputes or lack of proper medical verification. It emphasized that the State and medical professionals have a duty to protect life, and legalizing active euthanasia could lead to serious consequences and abuse. Thus, while passive euthanasia is permitted under strict conditions to allow a natural and dignified death, active euthanasia continues to be prohibited due to its direct involvement in causing death and the risks associated with it.

Constitutional Basis of Passive Euthanasia⁹

Under the Constitution, the most significant provision governing passive euthanasia is Article 21, which guarantees that "No person shall be deprived of his life or personal liberty except according to procedure established by law." Over time, this provision has evolved from a narrow guarantee against arbitrary deprivation of life into a comprehensive source of substantive rights, making it the cornerstone of euthanasia jurisprudence in India.

In cases such as *P. Rathinam v. Union of India*, *Gian Kaur v. State of Punjab*, *Aruna Shanbaug v. Union of India*, *Common Cause v. Union of India*, and the recent *Harish Rana Passive Euthanasia Case*, the interpretation of Article 21 has been central. These cases collectively demonstrate the gradual transformation of the right to life into a right that also protects dignity at the end of life.

Article 21 does not merely guarantee survival or continued biological existence. The Supreme Court has consistently interpreted it to include the right to live with dignity, which encompasses physical, mental, and emotional well-being. In this context, compelling a person to remain alive through artificial life-support systems, despite having no hope of recovery, may reduce life to a mere mechanical process and thereby violate the essence of dignity protected under Article 21.

The early constitutional debate began with *P. Rathinam*, where the Court briefly recognized the right to die as part of Article 21, treating it as a facet of personal liberty. However, this position was overruled in *Gian Kaur*, where the Constitution Bench clarified that while there is no fundamental right to die, the concept of a dignified death is not

⁹ ISSN: 2583-0538

inconsistent with Article 21. The Court observed that the right to life includes the right to live with dignity up to the end of natural life, thereby laying out the doctrinal foundation for passive euthanasia.

This foundation was operationalized in *Aruna Shanbaug*, where the Court, drawing from international jurisprudence such as *Airedale NHS Trust v. Bland*, permitted passive euthanasia under strict judicial and medical safeguards. The Court distinguished between active and passive euthanasia, holding that withdrawal of life support could be permissible when it is in the best interests of the patient and subject to judicial oversight. This case introduced the “best interest” test and the requirement of a medical board review, ensuring that the decision is not arbitrary or motivated by extraneous considerations.

The constitutional position was finally crystallized in *Common Cause*, where a Constitution Bench unequivocally held that the right to die with dignity is a fundamental right under Article 21. The Court recognized advance directives (living wills) as legally valid instruments through which individuals can exercise their autonomy regarding end-of-life decisions. This judgment harmonized multiple constitutional principles of dignity, autonomy, privacy, and bodily integrity into a coherent framework supporting passive euthanasia.

Further strengthening this framework is the recognition of the right to privacy in *K.S. Puttaswamy v. Union of India*, where the Court held that personal choices relating to one’s body and medical treatment fall within the protected sphere of privacy. This case is not based on euthanasia but includes the element of the importance of passive euthanasia which is End-of-life decisions, including the refusal of life-sustaining treatment, are thus considered deeply personal and constitutionally protected by choices.

Additionally, the doctrine of informed consent, affirmed in *Samira Kohli v. Dr. Prabha* ¹⁰

Manchanda, reinforces that no medical treatment can be administered without the patient’s consent. Passive euthanasia can therefore be understood as an extension of this doctrine, where the patient chooses to withdraw consent from continued medical intervention if there is no scope of further recovery and due to lessen the suffering and the pain of the patient with sufficient evidence court will grant further procedure

Article 14 must also be acknowledged in ensuring that decisions relating to passive euthanasia are not arbitrary. The procedural safeguards developed by the judiciary, such as medical board approvals and judicial oversight, are rooted in the principle of non-arbitrariness and fairness, which is a core component of Article 14.

More recently, cases such as the *Harish Rana* matter demonstrate the practical application of these constitutional principles. The courts have begun to implement the guidelines laid down in *Common Cause*, allowing withdrawal of life support in appropriate cases while maintaining strict procedural safeguards. This marks a shift from theoretical recognition to real-world enforcement of the right to die with dignity.

Thus, the constitutional basis of passive euthanasia in India lies in the expansive interpretation of

Article 21, read in conjunction with Articles 14 and 19, and supported by the principles of dignity, autonomy, privacy, and informed consent. It reflects a broader constitutional philosophy that values not only the preservation of life but also the quality, dignity, and meaningfulness of human existence until its natural end.

¹⁰ AIR 2008 SUPREME COURT 1385

Case analysis of Harish Rana vs Union Of India & Ors. on 2 July, 2024 ¹¹

Before going into the detailed analysis of the case, it is important to first understand the facts of the case. The case concerned Harish Rana, a 32-year-old man who suffered a severe brain injury after falling from the fourth floor of a building at his paying guest accommodation many years ago. The accident left him in a Persistent Vegetative State (PVS), a condition in which a patient loses awareness of surroundings but continues basic biological functions. Medical records showed that Rana had remained completely paralysed with 100% quadriplegia and had not shown any signs of recovery for over thirteen years. He survived only through Clinically Administered Nutrition (CAN), which was delivered through surgically inserted feeding tubes. According to the Court, medical reports consistently demonstrated that his condition had not improved and that the continuation of treatment merely prolonged biological existence without offering any real possibility of recovery. This situation clearly placed before the Court a case where life was being sustained only through artificial and mechanical means, thereby raising serious constitutional and ethical concerns regarding the quality and dignity of such existence.

If we move further, it was mentioned in paragraph 3 of the case that the family of the petitioner had consulted various doctors, all of whom informed them that there was no scope of recovery from the present condition. It was stated that the petitioner had not responded for the last 11 years and had developed deep and large bed sores, which further caused infection and deterioration in his health. It was also submitted that the petitioner's family had lost all hope for his recovery and were no longer in a position to take proper care of him due to their advancing age and physical limitations. The emotional, psychological, and financial burden on the family was also highlighted before the Court. It was in this situation that the petitioner approached the Court seeking a direction to refer the case to a Medical Board to determine whether he could be permitted to undergo passive euthanasia. This step was not merely a legal action but also reflected the humanitarian distress arising from prolonged suffering and helplessness.

The Court in this case was not only concerned with the question of whether passive euthanasia should be allowed, but also attempted to understand the broader surrounding circumstances, including the condition of the family members and the social realities attached to such cases. In paragraph 4, the Court carefully examined the fundamental question of whether a patient should be compelled to remain alive merely because medical technology is capable of sustaining life, or whether such a person should instead be allowed to die with dignity rather than continue in a meaningless existence filled with suffering. With reference to **Common Cause v. Union of India**, the Court addressed the conflict between medical ethics and human dignity. It analysed the concept of death in constitutional terms and raised the critical issue of whether a patient should be forced to endure suffering due to the rigid application of medical ethics, which obligates doctors to preserve life, even when the patient is unconscious and unaware of their surroundings. On the other hand, the Court considered whether permitting a dignified death would be more consistent with constitutional values.

At the same time, the Court recognised the role of societal pressure, which it described through the concept of “meaningful guilt”. This concept reflects the emotional dilemma faced by family members, who continue life-sustaining treatment out of a sense of moral obligation and fear of social judgment, even when they are aware that there is no hope of recovery. This results in prolonged suffering not only for the patient but also for the family, along with significant financial burden. Additionally, the Court addressed another important concern mentioned in paragraph 6, which is the possibility of misuse. It observed that in certain situations, beneficiaries may desire the patient's death in order to inherit property, thereby creating a risk of abuse. Due to such concerns, doctors are also hesitant and fearful

¹¹ 2026 SCC OnLine SC 358

of allegations of collusion, which may expose them to criminal liability and social stigma. These factors make the decision-making process highly complex and require a careful and balanced approach.

Furthermore, the Court undertook a detailed examination of the definition of euthanasia by analysing its historical and social context. It observed that although the Greek definition of euthanasia signifies “an easy and gentle death”, the term itself is a loaded one, and people have struggled with its meaning for a long time. The Court acknowledged that euthanasia does not have a universally accepted or philosophically consistent definition and has often been used as a tool of persuasive rhetoric. It is also defined as “killing at the request of the person killed”, which is the understanding adopted by Dutch medical authorities. However, the Court contrasted this with the misuse of the term in Nazi discourse, where euthanasia referred to the killing of individuals considered a burden on society, including for purposes such as eugenics, racial purity, and resource management. The Court firmly rejected such interpretations, stating that in modern democratic societies, these ideas are completely unacceptable. It emphasised that patients cannot be treated as burdens or “useless mouths”, and any such reasoning would violate constitutional principles of dignity. Instead, the focus must remain on the dignity of the individual and the constitutional recognition of the right to die with dignity under Article 21.

This discussion further led the Court to differentiate between the types of euthanasia, as stated in paragraph 218. In common parlance, euthanasia can be classified into three types: voluntary euthanasia, non-voluntary euthanasia, and involuntary euthanasia. Voluntary euthanasia refers to killing at the request of the person killed, where the patient consciously and voluntarily seeks termination of life, usually in cases of intolerable suffering or terminal illness, and such a request must be free from any external pressure. Non-voluntary euthanasia applies where the patient is incapable of making or refusing such a decision, and therefore, a proxy decision is made on their behalf. This form is particularly relevant to the medical profession, as it involves decision-making for patients who are unable to express their wishes. Involuntary euthanasia involves ending the life of a patient without their consent, even though they are capable of making such a decision, often based on a paternalistic assumption of what is best for the patient. The Court noted that such cases are highly problematic and raise serious ethical concerns, especially due to the potential for misuse and abuse of authority.

The Court further relied on earlier judgments such as **Gian Kaur v. State of Punjab** and **Aruna Shanbaug v. Union of India** to address important legal questions in the present case. It observed that in *Gian Kaur*, the Court had not clarified who could decide whether life support should be withdrawn in the case of an incompetent patient, such as a person in a coma or PVS. This created a significant gap in the law, particularly in the Indian context where such situations are not uncommon. The Court therefore emphasised the need for extreme caution, especially in light of the risk of misuse by individuals acting out of personal interests, including inheritance disputes.

The Court also pointed out that with rapid advancements in medical science, doctors must exercise caution before declaring a patient to be beyond recovery. It highlighted that a patient should not be considered a hopeless case unless there is absolutely no reasonable possibility of improvement in the near future through medical advancements. This was supported by the example of Terry Wallis, who regained consciousness after 24 years, demonstrating that medical recovery, although rare, is possible. However, the Court clarified that such assessments must be made by qualified medical experts.

Further, referring to **Airedale NHS Trust v. Bland**, the Court discussed the concept of brain death, defined as the irreversible cessation of all functions of the entire brain, including the brain stem. It noted that passive euthanasia may be permissible in cases where a person is clinically dead or where there is no meaningful interaction with the external environment.

The Court ultimately identified two key conditions under which passive euthanasia may be considered. First, when a person is kept alive solely through mechanical support without any consciousness, and second, when there is no reasonable possibility of recovery over a prolonged period. These conditions provide a practical framework for determining when withdrawal of life support may be justified.

Additionally, drawing guidance from **Vishaka v. State of Rajasthan**¹², the Court laid down procedural safeguards to ensure that decisions regarding passive euthanasia are taken with caution and responsibility. It clarified that the decision to withdraw life support may be taken by parents, spouse, or close relatives, or in their absence, by a “next friend” or even by the treating doctors, but such a decision must always be made in the best interest of the patient. In cases like *Aruna Shanbaug*, the Court recognized the hospital staff as the true “next friend” due to their continuous care and involvement.

Importantly, even if such a decision is taken, it must receive approval from the High Court concerned, as laid down in *Airedale* case, in order to prevent misuse and ensure that the rights of the patient are adequately protected.

At last, from the overall analysis of the case, it can be understood that the Court, apart from allowing passive euthanasia, has also clarified several important legal and ethical concepts which are essential in determining whether passive euthanasia should be permitted. It has strengthened the understanding of the right to die with dignity under Article 21, while simultaneously ensuring that adequate safeguards are in place to prevent misuse. This case therefore plays a significant role in guiding future judicial decisions and ensuring that such sensitive matters are addressed with care, compassion, and constitutional responsibility.

Future Implications and Need for Reform

The constitutional and legal understanding of passive euthanasia in India is primarily derived from judicial precedents and authoritative commentaries on constitutional law. The leading authority is *Common Cause v. Union of India*, reported in (2018) 5 SCC 1, where a Constitution Bench of the Supreme Court recognized that the right to die with dignity is an intrinsic part of the right to life under Article 21 and also gave legal recognition to advance directives or living wills. This judgment builds upon earlier decisions such as *Aruna Shanbaug v. Union of India*, (2011) 4 SCC 454, where the Court for the first time permitted passive euthanasia under strict judicial supervision and laid down procedural safeguards. The jurisprudential foundation was earlier shaped in *Gian Kaur v. State of Punjab*, (1996) 2 SCC 648, where the Court held that the right to life does not include the right to die, but recognized that the right to live with dignity includes a dignified process of death. This overruled the earlier view taken in *P. Rathinam v. Union of India*, (1994) 3 SCC 394, which had taken a contrary position by linking the right to die with Article 21.

In addition to these Indian precedents, comparative jurisprudence has also influenced the development of the law. The decision in *Airedale NHS Trust v. Bland*, [1993] AC 789, recognized the permissibility of withdrawing life-sustaining treatment from a patient in a persistent vegetative state, thereby providing persuasive guidance to Indian courts, particularly in *Aruna Shanbaug*. Further, the recognition of autonomy and bodily integrity as part of Article 21 in *K.S. Puttaswamy v. Union of India*, (2017) 10 SCC 1,¹³ strengthens the legal basis for allowing individuals to refuse medical treatment, which is a core aspect of passive euthanasia. The principle of informed consent, affirmed in *Samira Kohli v. Dr. Prabha Manchanda*, (2008) 2 SCC 1, further reinforces that medical intervention requires the patient’s consent, thereby supporting the withdrawal of treatment in appropriate cases.

¹² AIR 1997 SUPREME COURT 3011

¹³ (2017) 10 SCC 1

The courts have also relied on procedural innovations to fill legislative gaps, as seen in *Vishaka v. State of Rajasthan*, (1997) 6 SCC 241, where guidelines were framed in the absence of legislation; a similar approach was adopted in *Aruna Shanbaug* and later refined in *Common Cause* for regulating passive euthanasia. Authoritative constitutional commentaries such as M.P. Jain *Indian Constitutional Law* and V.N. Shukla's *Constitution of India* provide detailed academic analysis of Article 21 and its expansion to include dignity, autonomy, and end-of-life decision-making.

These sources collectively establish that the recognition of passive euthanasia in India is firmly grounded in Article 21 of the Constitution, interpreted through judicial precedents, supported by comparative law, and strengthened by academic scholarship. However, despite this well-developed jurisprudence, the absence of comprehensive legislation and the complexity of procedural safeguards continue to pose challenges for effective implementation.

Conclusion

The analysis of *Harish Rana Passive Euthanasia Case* clearly demonstrates the evolving constitutional position of passive euthanasia in India. The case not only applies the principles laid down in earlier landmark judgments such as *Common Cause v. Union of India* and *Aruna Shanbaug v. Union of India* but also deepens the understanding of the right to die with dignity under Article 21 of the Constitution. The Court has moved beyond a narrow interpretation of the right to life and has embraced a more humane approach that recognizes dignity, autonomy, and the quality of life as essential components of constitutional protection.

At the same time, the judgment reflects the inherent complexities involved in end-of-life decisionmaking. Issues such as the emotional and financial burden on families, the ethical obligations of medical professionals, and the potential for misuse make passive euthanasia a highly sensitive and challenging area of law. The Court has attempted to strike a careful balance by allowing passive euthanasia while imposing strict procedural safeguards to prevent abuse and ensure that decisions are made in the best interest of the patient.

However, despite these judicial advancements, significant gaps remain in the legal framework. The absence of comprehensive legislation, lack of awareness regarding living wills, and procedural delays continue to hinder effective implementation. Therefore, there is an urgent need for clear statutory guidelines, institutional support systems, and greater public awareness to ensure that the right to die with dignity is not merely theoretical but is meaningfully realized in practice.

In conclusion, the *Harish Rana* case represents an important step in the development of Indian constitutional jurisprudence. It reinforces the idea that the right to life under Article 21 includes the right to a dignified death, while also emphasizing the need for caution, compassion, and accountability in dealing with such deeply personal and ethical issues.

Suggestion

while passive euthanasia has now been firmly recognised in Indian constitutional law, its real-world application still reflects a gap between legal theory and practical accessibility. The existing framework, though well-intentioned, remains complex and often difficult for patients, families, and even medical professionals to navigate. There is a clear need for a more simplified and uniform procedural mechanism that can be implemented without unnecessary delays, especially in cases involving prolonged suffering. At the same time, greater awareness must be created among people regarding living wills and their legal validity, as many individuals remain unaware of this important right. Medical practitioners should also be provided clearer guidelines and legal protection so that decisions can be taken without fear

of liability. Further, a comprehensive legislation, rather than reliance solely on judicial guidelines, would help in bringing consistency and clarity across the country. Ultimately, the focus should remain on balancing safeguards against misuse with the fundamental idea of dignity, ensuring that the right to die with dignity is not merely recognised in law but is also meaningfully accessible in practice.

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